

1. After review of the statement of work we see that it mentions all the repairs are needed from the previous report but it doesn't share any of those details. The only details it shares is replacing some old check valves and valves on the oxygen main line. Could you please clarify what repairs are being requested?

Response: See A0002_Attachment B

2. Please clarify if the project will require a full time:
 - QA/QC (Full Time/Part Time?)

Response: Full Time / Can Have Other Duties

- SSHO (Full Time/Part Time?)

Response: Full Time / No Other Duties

3. Please confirm the anticipated allowable construction working hours for this project, including any restrictions related to weekdays, weekends, or after-hours work.

Response: Primary work schedule will be normal business hours (Monday through Friday, 7:00AM to 4:00PM Central Time). The conduit installation in the surgery department will need to be completed after hours. Please see 001.T-120. Work at the SICU can be completed during normal business hours but no later than 6:00 PM due to quiet / sleep hours. Work in the OR pre-op holding and sterile equipment space can be done during normal business hours.

4. Can an asbestos survey of existing spaces be provided?

Response: Yes, please see A0002_Attachment C. Only JLM has been surveyed. VA is not anticipating any issues with asbestos in either building.

5. Will the VA be responsible for removal of the furniture or medical equipment from the space prior to construction, or will that be the contractor's responsibility?

Response: The VA does not anticipate any furniture or medical equipment needing to be removed from the space.

6. Does the Table of Contents count towards the Volume I -Past Performance page limitations?

Response: No

RFIs - 36C25626R0006 - Med Gas Deficiencies
598-23-107

7. Does the Offer Letter count towards the Volume I -Past Performance page limitations?

Response: Yes

8. Does a Firm Overview page count towards the Volume I -Past Performance page limitations?

Response: Yes

9. Do the CPARS or completed Past Performance Questionnaires count towards the Volume I -Past Performance page limitations?

Response: No

10. If we have a previously completed Past Performance Questionnaire, requested for a different opportunity, can we use it in lieu of requesting new ones to be completed?

Response: PPQs were not requested to be submitted for this solicitation; therefore, there is no need to submit.

11. Can we use a previously completed version of the Experience Modification Rate form (as one was not provided with the solicitation documents)?

Response: See attachment A0002_Attachment A - EMR Form for completion.

12. Do you require a completed FAR Clause 52.204-8? If so, do you want it to be returned inside the Volume II - Price Proposal?

Response: Yes

13. Do you require a completed FAR Clause 52.204-24? If so, do you want it to be returned inside the Volume II - Price Proposal?

Response: Yes

14. Do you require a completed FAR Clause 52.209-7? If so, do you want it to be returned inside the Volume II - Price Proposal?

Response: Yes

15. Do you require a completed FAR Clause 52.209-13? If so, do you want it to be returned inside the Volume II - Price Proposal?

Response: Yes

RFIs - 36C25626R0006 - Med Gas Deficiencies
598-23-107

16. Do you require a completed FAR Clause 52.219-28? If so, do you want it to be returned inside the Volume II - Price Proposal?

Response: Yes

17. Do you require a completed FAR Clause 52.225-9? If so, do you want it to be returned inside the Volume II - Price Proposal?

Response: No

18. Please confirm whether the VA will provide temporary oxygen during required oxygen valve replacement shutdowns at JLM and EJT hospitals, or if temporary oxygen is the contractor responsibility.

Response: Yes, VA will provide. A 60-day notice is required before equipment needing temporary oxygen supplies can be taken offline.

19. Will the VA confirm whether service/zone valves exist at each floor or within critical care areas at JLM Hospital to allow backfeeding during the temporary shutdown for valve replacement? IF SO identify where service/zone valves currently exist.

Response: Yes, the valves are located in hallway of each POD.

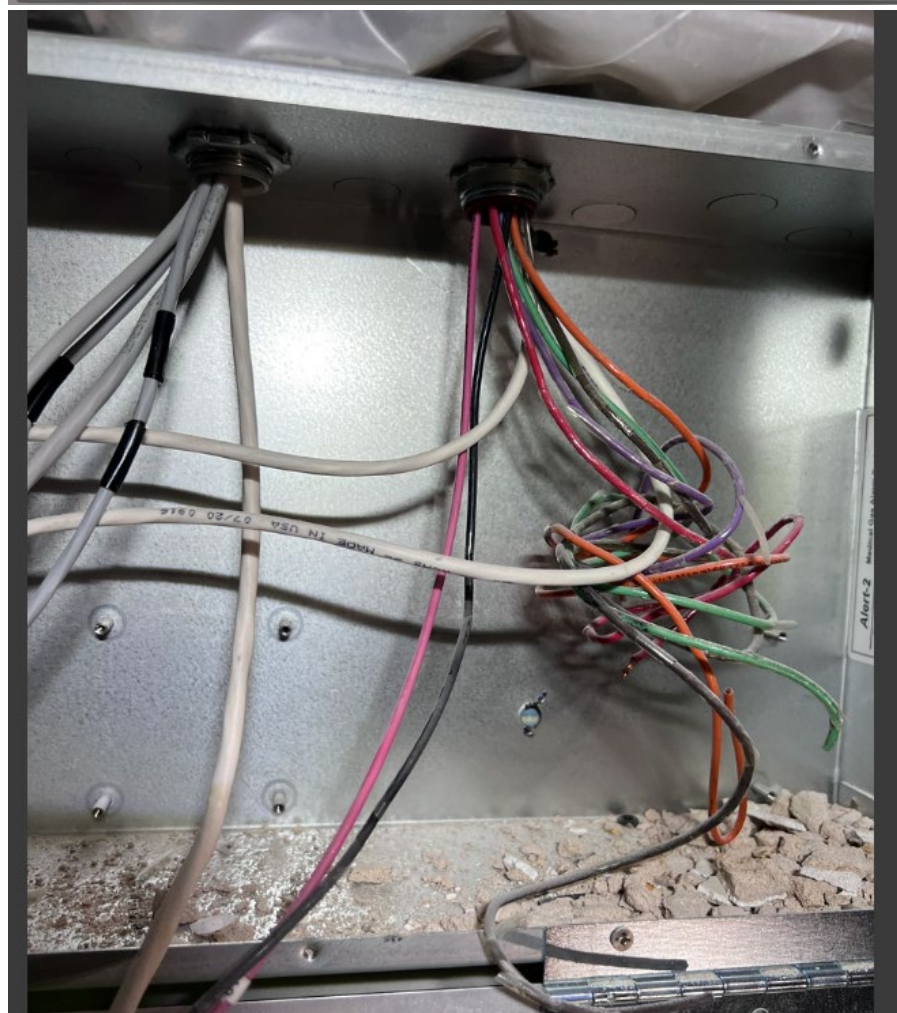
20. Will the VA confirm whether service/zone valves exist at each floor or within critical care areas at EJT Hospital to allow backfeeding during the temporary shutdown for valve replacement? IF SO identify where service/zone valves currently exist.

Response: Yes, valves are located in each unit's main corridor.

21. Will the VA provide photos of the following master and area alarm locations at JLM Hospital that were not accessible during the walkthrough:

- a. Existing master alarm MA-SPS.
- b. New master alarm panel located in the Plumbing Shop (GD114F).
- c. Area alarm serving Pre-OP / Recovery Rooms 1–20, in Wheelchair Storage Room 2W157M.
- d. Existing master alarm located in Reception Room 2B101A ?
- e. Area alarms serving Rooms 1–12 and 16–27 on the 5th Floor

a. Response:



RFIs - 36C25626R0006 - Med Gas Deficiencies
598-23-107

- b. Response: The new alarm panel in the plumbing shop is to be provided under this project. A picture of the proposed location has been provided.**



RFIs - 36C25626R0006 - Med Gas Deficiencies
598-23-107

c. **Response:**



RFIs - 36C25626R0006 - Med Gas Deficiencies
598-23-107

d. **Response:**



RFIs - 36C25626R0006 - Med Gas Deficiencies
598-23-107

e. **Response:**



22. Please refer to Drawing 001.T-150, Key Note 1, which references replacing gas specific transducers only. Will the VA confirm whether, in this area of the hospital, the scope includes replacement of area alarm conversion kits, or if the work is limited to transducer replacement only?

Response: Replacement of area alarm with conversion kits. All of the conversion kits need to have the same type of transducers that will be used everywhere.

23. Have any parts of the medical gas alarm systems at JLM/EJT Hospital been recently renovated or upgraded? If so, please identify the specific floors and locations, and clarify which components were updated or replaced.

Response: All items/equipment indicated on plans to be new.

24. Please refer to 001.T-120 for the existing area alarm noted for relocation in the Surgical Area, Second Floor. Will the VA confirm if a new area alarm assembly is required versus reusing the existing back box with replacement of the conversion kit and transducers.

Response: New is required.

25. Will the VA provide the electrical as-built drawings identifying the emergency power panels that will serve the new medical gas master alarms at EJT and JLM Hospitals, including room numbers and approximate locations?

Response: Approximate locations are noted in the design drawings. A map for GCL1 has been provided. Please see A0002_Attachment D.

26. If possible, please also provide photos of the emergency panels to verify available spare breaker spaces.

- a. Drawing 001.T-100, Key Note 25 - "Provide new 120V circuit from existing panel 'GCL1'." Please identify panel location and room number.
- b. Drawing 170.T-100, Key Note 14 - "Provide new 120V circuit from existing panel 'EL/GE'." Please identify panel location and room number.
- c. Drawing 170.T-120, Key Note 4 - "Provide one (1) 20-amp , single pole circuit breaker with KAIC rating matching the power panel." To ensure compatibility, please provide the bra manufacturer and type and required KAIC rating for Panel 'EL/2B'?

Response: Please see A0002_Attachment E. Breakers in EL/2B are 22 KAIC.

27. Please refer to 001.T-120 Detail 1, Key Note 9 which states: "Carbon dioxide cabling from the first floor to the second floor using existing penetrations." However, the associated photo callout indicates an area alarm serving the SICU. Please confirm

whether this is a note error and clarify if the VA intends to replace the area alarm conversion kits serving the SICU.

Response: Key note 9 is supposed to say, “EXISTING AREA ALARM SHALL BE REPLACED WITH A CONVERSION KIT. PROVIDE GAS SPECIFIC TRANSDUCERS AND CABLING.”

28. Drawing 001.P-400, Key Note 5, states: “Replace existing shutoff valve in auxiliary valve box with new.” However, replacing just the shutoff valve is labor-intensive and time-consuming due to the current configuration.

- a. Will the government revise this requirement and accept the replacement of the entire emergency oxygen inlet station (EOSC) rather than just the shutoff valve?
- b. If the requirement is revised to replace the entire EOSC, current code requires integration of the temporary oxygen connection with the master alarm system, including provision of a total of (12) 2 pair shielded twisted cables. Will the VA include this requirement in the solicitation?

Response: Replace the valve only.

29. Drawing 001.P-400, Key Note 3: Will the VA accept the installation of the new lockable shutoff valve next to the auxiliary valve box? This location would provide easy access for maintenance, eliminate the need to install the valve at an elevated height, and allow personnel to operate it without entering the building.

Response: No, please follow current design.

30. Will the VA provide any/ all as-built drawings showing the medical gas piping for both EJT and JLM Hospitals?

Response: Please see A0002_Attachment F.

31. Drawing 170.P-400, Key Note 1: Please confirm whether there is an isolation valve in the oxygen main line that would allow the EOSC to backfeed the facility during check valve replacement?

Response: There is not an isolation valve to allow backfeeding the facility from the EOSC during check valve replacement.

32. Will the VA accept, as an alternative at the EJT campus, installing a new underground communication line between Bldg 170 and Bldg 69 through horizontal boring? This alternative would eliminate the need for a 7-foot-deep excavation alongside a high-traffic area.

Response: All underground conduit must be encased in concrete, and must meet all other requirements of the contract documents. Means and methods are up to the contractor.

33. Will the VA accept horizontal boring as an alternative method for installing the new underground communication line at the Little JLM campus? This approach would eliminate the need to relocate existing mechanical equipment.

Response: Means and methods are at the discretion of the contractor. If bore and receiving pits can be installed without interrupting traffic flow, or the VA approves the interruption, the civil engineer has no issue with boring. The underground conduit must also be concrete encased and meet all other contract performance requirements.

34. Please refer to Drawing CU101, Key Note 2. This note states to proposed a 24" x 30" pull box to allow for the repair and connection of the broken conduit and wiring. Please refer to Drawing TS101, Key Note 3. This note states the installation of a doghouse-style manhole over the broken duct. There is a discrepancy between these two notes. Will the VA please clarify which option the contractor is required to provide?

Response: Civil – Any structure rated Tier 15 that allows for the repair of broken conduit and installation of new cabling is acceptable.

35. Drawing 170.T-100, Key Note 5, indicates use of the existing J-Box. Will the VA allow installation of a new J-Box to reduce alarm downtime during the switchover?

Response: That is acceptable.

36. Drawing 170.T-100, Key Note 5. During the walkthrough, the J-Box was not accessible. Will the VA provide a clear picture of the junction box with the cover open?

Response:



37. For the EJT Hospital, will all alarm wiring and conduit installation be performed during normal business hours and the normal work week?

Response: Yes.

38. For the JLM Hospital, will all alarm wiring and conduit installation be performed during normal business hours and the normal work week?

Response: Yes.

39. For the EJT Hospital, will termination of new alarm cabling at the master alarm panel be performed during normal business hours and the normal work week?

Response: Yes.

40. For the JLM Hospital, will termination of new alarm cabling at the master alarm panel be performed during normal business hours and the normal work week?

Response: Yes.

41. For the JLM and EJT Hospitals, will all alarm wiring and conduit installation be required to be performed using a containment cart or other type of device?

Response: Yes. There will be areas and type of work where containment carts and methods will be required. This is particularly true in the OR area and other patient areas where there will be cutting in walls.

42. For the JLM and EJT Hospitals, will the Contractor be required to install ICRA barriers for the replacement of the area alarms?

Response: It depends on the type of construction and risk group of the area of the hospital. See G-005 for guidelines.

43. Will the VA consider a Value Engineering option for master alarm panel alarm wiring installed in accordance with NFPA 99? The proposed alarm wiring would be code-compliant and shielded.

Response: VA requires conduit for new cabling.

44. Drawing CU101, Utility Note 5, references utility mapping and scanning exhibits (McCarthy Mapping “butterfly diagrams” and Elite Scanning report dated February 26, 2024). Please confirm whether these documents are part of the contract documents and if so, will the VA provide copies for review?

Response: Please see A0002_Attachment G.

45. For the JLM and EJT campuses, please confirm any restricted work hours, noise limitations, or requirements for night or weekend work affecting excavation, concrete placement, or restoration activities.

Response: No noise restrictions for that portion of the work.

46. For the JLM and EJT campuses, please confirm the required pavement and concrete restoration limits for this project (minimum trench patching versus full panel or lane replacement).

Response: Minimum trench patching for asphalt per CU501. Trench patching for the concrete on the JLM site is acceptable.

47. For the JLM and EJT campuses, please confirm whether VA/COR approval is required prior to concrete placement for duct banks, manholes, or encasements, and identify typical review and approval durations.

Response: Yes, AE will review submittals according to VA established review times.

48. Installation of the communication duct bank as shown on Drawing CU100 would require relocation of the existing chiller, which would impact hospital operations. Will the VA confirm whether installation beneath the chiller is acceptable, and if so, provide the required minimum clearance, depth, encasement requirements, and acceptable excavation methods (e.g., hydrovac)?

Response: The chiller is a temporary unit. The unit is in that location between 04/01 – 10/30 each year.

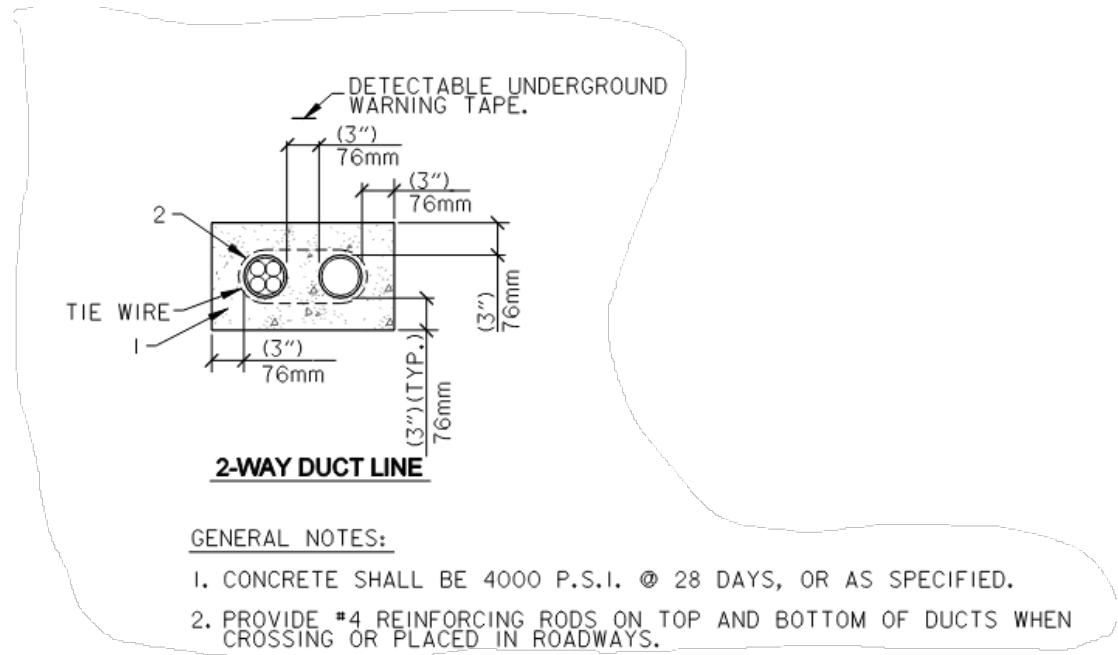
49. If installation beneath the chiller is not acceptable, will the VA provide an alternate route for the communication line where routing beneath existing mechanical equipment is permitted? If so, please provide the required minimum clearance, depth, encasement requirements, and acceptable excavation methods (e.g., hydrovac).

Response: See response to Q48 above.

50. For the JLM campus, the drawings specify the number and size of conduits but do not provide details for the communication duct bank configuration. Will the VA

provide the duct bank concrete encasement dimensions and strength, as well as the minimum cover depth?

Response: Here is a standard concrete duct bank detail.



51. Drawing 001.T-100 references installation underground communication lined duct. Please confirm this scope is limited to communications conduits only, with no oxygen or medical gas piping included.

Response: 001.T-100 is only for underground communication and not oxygen or medical gas piping.

52. Drawing CU100, Key Note 10, states "Provide new 12" x 17" handhole/pull box." Will the VA confirm the acceptable manufacturer(s), model(s), and required load rating for the specified handhole?

Response: Please provide traffic rated (Class D).

53. Drawing CU100, Key Note 10. Please confirm required load/traffic rating and dimensions for the specified heavy-traffic-rated handhole.

Response: See response to Q52 above.

54. Drawing CU100, Key Note 5, indicates removal of existing underground electrical. Will the VA confirm the specific segments to be removed, verify de-energization requirements, and clarify disposal requirements?

Response: Line is to be removed in its entirety and disposed of offsite. Install new parallel to existing. De-energize and remove existing after new has been installed, tested and the VA accepts the new line.

55. For the EJT campus underground utility work, will the VA confirm the required method and standard for repair and replacement of the existing communications duct bank, including conduit type, encasement requirements, and connection details to the existing infrastructure?

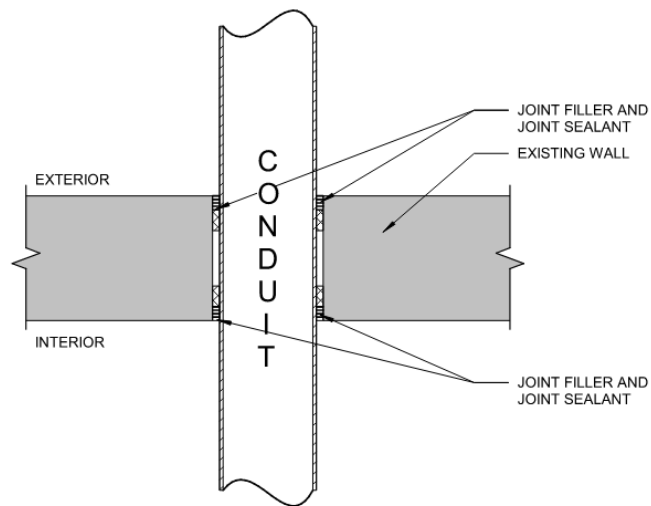
Response: It will be the contractor's responsibility to provide means and methods. Please see specifications for equipment and material information.

56. Will the VA provide as-built drawings showing the details of the existing communications duct bank at the EJT campus?

Response: These drawings are not available

57. Drawing 001.T-100, Key Note 13. Please confirm the accepted waterproofing method/material, sleeve requirements, and firestopping expectations (if applicable) for communications conduit penetrations through exterior building walls.

Response: Please see specifications page 07 92 00-5 Part 2 Products 2.1 Sealants A. Exterior Sealants.



12 CONDUIT WALL PENETRATION DETAIL
Scale: 3" = 1'-0"

58. Per Specification Section 019100, Para. 1.9, has the VA appointed an individual, company, or firm as the Commissioning Agent (CxA) for this project?

- a. If so, please provide the name of the appointed individual, company, or firm.
- b. If no, is this the VA's responsibility or should offerors include the cost of a CxA in our price proposals?

Response: Nathan Tyler with Med Gas Testing Services is the Commissioning Agent.

59. Will the VA provide an extension of the bid due date by at least two weeks after all RFI's responses are published to allow sufficient time to review and prepare them to provide the best price possible?

Response: RFP due date is extended to February 3, 2026, at 4:00PM Central Time.

60. Will a material storage or staging area on JLM and EJTB campuses be available for the contractor to store equipment and materials, as well as to receive deliveries?

Response: Staging area at EJTB is North Lot 10 and the staging area at JLM will be the grounds building.

61. Will the contractor be required to provide a surgically sterile clean environment upon completion of the renovation, or only final construction cleaning as typically required under standard construction contracts, given that new wiring will be installed through surgical corridors and sterilization rooms?

Response: The contractor is required to provide a sterile space in patient care areas and final construction cleaning for common areas.

62. Please confirm if the Superintendent can share the role of CQM on this project?

Response: The Superintendent may share the role of CQM as long as they qualify per the project specs.

63. Will the government allow the Prime Contractor to delegate an OSHA 30 Superintendent and associated responsibilities to a member of the subcontractor's team?

Response: Prime shall provide the SSHO.

64. Will the Contractor be required to provide a full-time on-site Superintendent, CQM, and SSHO, for this project?

- a. Are all 3 roles required to be separate personnel or can duties overlap?
- b. If duties can overlap, please indicate which roles can overlap.
- c. Can any of these personnel be assigned to any other projects?

Response: SSHO must be full-time with no other duties. Specification says The CQC system manager is on site all time and has duties as project superintendent in addition to quality control. If any of these personnel are dedicated full time, then they cannot be assigned to any other projects.

65. Will the government clarify if a TB test is required for all on-site personnel, including the Prime Contractor and any subcontractors?

Response: A TB test is required for all on-site personnel.

66. Has an asbestos and lead survey been conducted in the JLM Hospital mechanical rooms GD133, GE12, and GF143? If so, will the VA provide copies of the survey results?

Response: Yes, please see A0002_Attachment C. Only JLM has been surveyed. The VA is not anticipating any issues with asbestos in either building.

67. Has an asbestos and lead survey been conducted in the EJT Hospital mechanical rooms GJ103, GB111, and the boiler plant? If so, will the VA provide copies of the survey results

Response: Yes, please see A0002_Attachment C. Only JLM has been surveyed. The VA is not anticipating any issues with asbestos.

68. Will the government consider allowing the awarded contractor some space inside the work site or in the immediate area surrounding the site to set up a desk for the onsite superintendent? This will allow him/her to have constant presence inside the facility and the work site.

Response: If space becomes available for contractor's use, yes, but there is no guarantee. Potential for use of an empty office on 3rd Floor at JLM if available.

69. For the JLM and EJT campuses, please confirm any restricted work hours, noise limitations, or requirements for night or weekend work affecting activities inside the hospital.

Response: Primary work schedule will be normal business hours (Monday through Friday, 7:00AM to 4:00PM Central Time). The conduit installation in the surgery department will need to be completed after hours. Please see 001.T-120. Work at the SICU can be completed during normal business hours but no later than 6:00 PM due to quiet / sleep hours. Work in the OR pre-op holding and sterile equipment space can be done during normal business hours.

70. Drawings 001.T-100, Keynote #18 and 170.T-100, Keynote #10 indicate to “run all new cabling in conduit, above the ceiling, from source equipment to the master alarm.” Per NFPA 99, wiring installation requirements reference and defer to applicable fire and electrical codes and the standard NFPA 70 (NEC). NEC Article 300.22 permits specific wiring methods within plenum spaces, including the use of plenum-rated cables without metal raceway. Will the VA allow the installation of shielded twisted-pair, plenum-rated cable within plenum spaces, in lieu of

conduit, from source equipment to the Master Alarm Panel? This approach would reduce installation duration and minimize disruption to hospital operations.

Response: VA standards require that Medical Gas Cabling run in conduit.

71. Specification Section 226300, Para. 2.4.A states: "Provide all low voltage control wiring, including wiring from alarm relay interface control cabinet to BAS, required for a complete, properly functioning system." However, the drawings do not indicate any BAS connections for the Master Alarm at either hospital. Will the VA please clarify whether the master alarms at JLM and EJT Hospitals are required to be connected to each hospital's BAS? If so, will the VA provide additional details for the required controls scope?

Response: Connecting MedGas panels to BAS systems is not required under this contract.